

D439

Foundations of Nursing

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 68-year-old retired teacher admitted two days ago for pneumonia tells the nurse she suddenly feels “short of breath and dizzy.” Her daughter, who is a pharmacist, is visiting. What should the nurse do first?

- A. Call the provider to request a new medication order.
- B. Ask the daughter what medication might be causing it.
- C. Increase her oxygen flow rate right away.
- D. Assess her oxygen saturation, respiratory rate, and lung sounds.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A nurse has just given a PRN pain medication. Which step of the nursing process is the nurse performing 30 minutes later when she asks the patient to rate his pain again?

- A. Implementation
- B. Evaluation
- C. Planning
- D. Assessment

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

On a busy medical unit staffed with two RNs and one experienced nursing assistant who has worked there eight years, which task is appropriate for the RN to delegate to the assistant?

- A. Performing the admission assessment on a new patient.
- B. Anything the assistant has done many times, including evaluating a patient’s response to a new medication.
- C. Measuring vital signs on a stable patient admitted yesterday.
- D. Teaching a newly diagnosed diabetic patient how to use a glucometer.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 45-year-old patient scheduled for a knee replacement at 9 a.m. has signed the consent form. As the nurse prepares him, he says, "I'm still not sure what they're going to do to my knee." His wife says the surgeon is the best in the state. What is the nurse's best action?

- A. Hold the pre-op preparation and notify the surgeon to re-explain the procedure.
- B. Proceed, because the consent form is already signed.
- C. Explain the surgical procedure and its risks to him in detail.
- D. Reassure him that the surgeon is highly skilled.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A patient who recently moved from overseas and works as a chef is admitted with suspected pulmonary tuberculosis. He has had a cough for three weeks. Which precautions should the nurse set up?

- A. Standard precautions only, because he has been coughing for weeks.
- B. Airborne precautions: negative-pressure room and a fitted N95 respirator.
- C. Contact precautions with gown and gloves.
- D. Droplet precautions with a surgical mask within 3 to 6 feet.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A nurse is setting up a room with a negative-pressure airflow system and N95 respirators. Which diagnosis is this setup meant for?

- A. Influenza
- B. MRSA wound infection
- C. Varicella (chickenpox)
- D. Clostridioides difficile

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A patient taking a loop diuretic for heart failure, who enjoys gardening and walks daily, has a serum potassium of 3.0 mEq/L this morning. The unit is short one nurse. Which statement about this result is correct?

- A. It is within normal limits.
- B. It is high and the diuretic should be continued to remove potassium.
- C. It can wait until the next scheduled lab draw.
- D. It is low and puts the patient at risk for dysrhythmias; the nurse should report it.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A patient with a new cancer diagnosis, whose son is flying in from another state tonight, says quietly, "I don't know how I'm going to get through this." Which response by the nurse is most therapeutic?

- A. "Why do you feel that way?"
- B. "At least your son will be here tonight."
- C. "Don't worry, the treatments today are very successful."
- D. "You're feeling unsure about what's ahead. Tell me more about what worries you."

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

Who is responsible for explaining a surgical procedure's risks and alternatives and obtaining informed consent?

- A. The provider performing the procedure
- B. The nurse who witnesses the signature
- C. The patient's next of kin
- D. Whoever on the team is available

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

Which nurse response is a barrier to therapeutic communication?

- A. "Everything will be fine; you have nothing to worry about."
- B. "Go on, I'm listening."
- C. "You seem worried. What's on your mind?"
- D. Sitting quietly with the patient after bad news.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A new graduate on a cardiac step-down unit that uses electronic charting reads the note “HR 48, sinus bradycardia.” The unit has 24 beds. What does bradycardia describe?

- A.** An irregular heart rhythm
- B.** Inflammation of the heart
- C.** A slow heart rate
- D.** A fast heart rate

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

An 82-year-old patient with a DNR order, a former engineer who likes to do crossword puzzles, develops a fever of 38.9 °C (102 °F) and says he is uncomfortable. What should the nurse do?

- A.** Withhold all treatment because he has a DNR order.
- B.** Assess him and give the ordered antipyretic and comfort measures.
- C.** Begin CPR preparations in case he worsens.
- D.** Ask the family to decide whether he should get acetaminophen.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 68-year-old ~~retired teacher~~ admitted two days ago for pneumonia tells the nurse she suddenly feels “short of breath and dizzy.” ~~Her daughter, who is a pharmacist, is visiting.~~ What should the nurse do first?

Target word: first action **Decides:** New complaint = assess before acting.

A	Dropped element	Skips the assessment data the provider will need.
B	Camouflage	Answers the pharmacist detail; the daughter is not the nurse’s assessment source.
C	Inverted operation	Implements before assessing; the order may not be indicated.
D	Correct	Assessment comes first so the nurse knows what is happening before choosing an action.

2. Answer B Concept 1 • transfer

A nurse has just given a PRN pain medication. Which step of the nursing process is the nurse performing 30 minutes later when she asks the patient to rate his pain again?

Target word: reverse: name the step from the action **Decides:** Checking whether an intervention worked is evaluation.

A	Inverted operation	Giving the medication was implementation; checking the result comes after it.
B	Correct	Rechecking the outcome after an intervention evaluates its effect.
C	Confusion set	Planning sets goals before action, not after.
D	Adjacent category	Close, but the purpose here is to judge the effect of a completed intervention.

3. Answer C Concept 2

On a busy medical unit staffed with two RNs and one experienced nursing assistant ~~who has worked there eight years~~, which task is appropriate for the RN to delegate to the assistant?

Target word: delegable task **Decides:** Routine care of a stable patient with predictable results.

A	Inverted operation	Assessment cannot be delegated.
B	Camouflage	Experience does not move evaluation into the assistant’s scope.
C	Correct	Routine, predictable task on a stable patient.
D	Adjacent category	Teaching belongs to the RN.

4. Answer A Concept 3

A 45-year-old patient scheduled for a knee replacement at 9 a.m. has signed the consent form. As the nurse prepares him, he says, “I’m still not sure what they’re going to do to my knee.” ~~His wife says the surgeon is the best in the state.~~ What is the nurse’s best action?

Target word: action when consent understanding is in doubt **Decides:** The patient does not understand the procedure; consent is not truly informed.

A	Correct	The provider must ensure understanding; the nurse advocates by stopping and notifying.
B	Absolute language	A signature does not make consent informed.
C	Adjacent category	Explaining the procedure and risks is the provider’s role.
D	Camouflage	Uses the wife’s comment; reputation does not fix a gap in understanding.

5. Answer B Concept 4

A patient who ~~recently moved from overseas and works as a chef~~ is admitted with suspected pulmonary tuberculosis. ~~He has had a cough for three weeks.~~ Which precautions should the nurse set up?

Target word: precaution type for TB **Decides:** TB spreads by airborne particles.

A	Dropped element	Drops the transmission route; symptom duration does not reduce risk.
B	Correct	TB nuclei stay suspended in air; airborne precautions are required.
C	Adjacent category	Contact precautions fit C. diff or MRSA, not TB.
D	Confusion set	Droplet precautions fit influenza, not airborne TB.

6. Answer C Concept 4 • transfer

A nurse is setting up a room with a negative-pressure airflow system and N95 respirators. Which diagnosis is this setup meant for?

Target word: reverse: from setup to disease **Decides:** Negative pressure plus N95 = airborne.

A	Confusion set	Influenza uses droplet precautions.
B	Inverted operation	Contact route, the opposite of the airborne setup.
C	Correct	Varicella is spread by the airborne route.
D	Adjacent category	C. diff needs contact precautions and soap-and-water hand hygiene.

7. Answer D Concept 5

A patient taking a loop diuretic for heart failure, ~~who enjoys gardening and walks daily~~, has a serum potassium of 3.0 mEq/L this morning. ~~The unit is short one nurse.~~ Which statement about this result is correct?

Target word: interpret K+ value **Decides:** 3.0 is below the 3.5–5.0 normal range; loop diuretics waste K+.

A	Wrong stem number	3.0 is below the 3.5 lower limit.
B	Inverted operation	The value is low; loop diuretics lower potassium further.
C	Camouflage	Staffing does not change the urgency of a cardiac risk.
D	Correct	Hypokalemia can cause dangerous heart rhythms and is expected with loop diuretics.

8. Answer D Concept 6

A patient with a new cancer diagnosis, ~~whose son is flying in from another state tonight~~, says quietly, “I don’t know how I’m going to get through this.” Which response by the nurse is most therapeutic?

Target word: therapeutic response **Decides:** Invite the patient to express feelings.

A	Adjacent category	“Why” questions can feel judgmental and put the patient on the defensive.
B	Camouflage	Redirects to the son’s visit instead of the patient’s concern.
C	Confusion set	False reassurance shuts down the feeling.
D	Correct	Reflects the feeling and uses an open-ended invitation.

9. Answer A Concept 3 • transfer

Who is responsible for explaining a surgical procedure's risks and alternatives and obtaining informed consent?

Target word: reverse: whose duty, not what the nurse does **Decides:** Consent is part of the provider's duty to inform.

A	Correct	The person doing the procedure must inform the patient and obtain consent.
B	Inverted operation	The nurse witnesses; the nurse does not supply the informing.
C	Adjacent category	Family consents only when the patient lacks capacity and a surrogate is needed.
D	Absolute language	Consent is not a task any team member may take on.

10. Answer A Concept 6 • transfer

Which nurse response is a barrier to therapeutic communication?

Target word: reverse: pick the non-therapeutic response **Decides:** False reassurance blocks the patient's feelings.

A	Correct	False reassurance dismisses the concern and ends the conversation.
B	Confusion set	A general lead that encourages the patient to continue.
C	Inverted operation	This is therapeutic: it reflects and invites.
D	Adjacent category	Therapeutic silence gives the patient time to process.

11. Answer C Concept 7

A new graduate on a cardiac step-down unit ~~that uses electronic charting~~ reads the note "HR 48, sinus bradycardia." ~~The unit has 24 beds.~~ What does bradycardia describe?

Target word: meaning of the term **Decides:** Prefix brady- = slow; -cardia = heart.

A	Adjacent category	Irregularity is dysrhythmia; the term only says slow.
B	Confusion set	That would use -itis (carditis).
C	Correct	Brady- means slow; cardia refers to the heart.
D	Inverted operation	That is tachycardia; tachy- means fast.

12. Answer B Concept 8

An 82-year-old patient with a DNR order, ~~a former engineer who likes to do crossword puzzles~~, develops a fever of 38.9 °C (102 °F) and says he is uncomfortable. What should the nurse do?

Target word: care allowed under DNR **Decides:** DNR limits CPR only.

A	Absolute language	DNR means no CPR, not no care.
B	Correct	DNR does not stop symptom management or other ordered treatment.
C	Inverted operation	The order says the opposite: no CPR.
D	Dropped element	Drops the fact that the patient is alert and ordered care should continue.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	9	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Nursing process order	Assess before you plan or act: Assessment → Diagnosis → Planning → Implementation → Evaluation.	Jumping to an intervention before gathering data.	A new or changed finding and an option that acts before checking.
2	Scope of practice and delegation	The RN may delegate stable, routine tasks to assistive personnel but keeps assessment, teaching, evaluation, and unstable patients.	Delegating anything a UAP has done before, including first-time teaching or assessment.	“Which task can the nurse assign to the nursing assistant?”
3	Informed consent	The provider explains the procedure and obtains consent; the nurse witnesses the signature and confirms the patient understands and is competent.	Believing the nurse explains the risks or that a signed form ends the discussion.	A patient with questions or doubts right before a procedure.
4	Transmission-based precautions	Match precautions to the route: airborne (TB, measles, varicella) needs a negative-pressure room and N95; droplet (influenza) needs a mask within close range; contact (C. diff, MRSA) needs gown and gloves.	Using a surgical mask for airborne disease or treating all isolation the same.	A named organism plus a question about room, mask, or PPE.
5	Potassium balance	Normal serum K ⁺ is about 3.5–5.0 mEq/L; both low and high levels threaten the heart, and IV potassium is never given by IV push.	Treating a potassium value as minor or assuming a bolus fixes it faster.	A potassium lab value, cardiac rhythm, or IV potassium order.
6	Therapeutic communication	Use open-ended, feeling-focused responses; avoid false reassurance, “why” questions, advice, and changing the subject.	Thinking reassurance (“everything will be fine”) is supportive.	A patient expresses fear or emotion and the options are nurse responses.
7	Medical terminology	Build meaning from parts: prefix + root + suffix (e.g., brady- slow, tachy- fast, -itis inflammation, -ectomy removal, -otomy incision).	Guessing from the root alone and ignoring the prefix or suffix.	An unfamiliar term that can be broken into word parts.
8	DNR and end-of-life care	A do-not-resuscitate order withholds CPR only; comfort, symptom management, and other ordered care continue.	Thinking DNR means “do not treat” or stopping routine care.	A DNR patient with a new symptom or care need.

Course scope (public description): Foundations of Nursing introduces students to the nursing process, scope of practice, clinical judgment model, and fundamental concepts of holistic nursing practice that will serve the needs of diverse adult patients across the lifespan. The course will focus on medical terminology, legal/ethical issues, basic care and comfort, oxygenation, safety & infection control, health and wellness, fluid & electrolytes, death and dying, therapeutic communication, patient education & advocacy.